

Western Tumor Center

Dept. of Oncology
55 University Way, Albuquerque, NM 87102

Discharge Summary

Patient:	Ewald Baumann
Date of Birth:	03/16/1944
Admission Date:	04/14/2025
Discharge Date:	04/23/2025
Attending Physician:	Prof. Martin Schuler, MD

Mr. Ewald Baumann was admitted for laparoscopic left nephrectomy. Histology revealed clear cell renal carcinoma, pT2a N0 M0.

Primary Diagnosis: Renal cell carcinoma (C64) — Tumor staging: T2a N0 M0

Laboratory values:

Parameter	Value	Reference
HbA1c	6.2%	< 6.5%
Creatinine	1.4 mg/dL	0.7–1.2 mg/dL
CRP	8.2 mg/dL	< 0.5 mg/dL

Discharged 04/23/2025.

Relevant Medical History:

The patient has a long-standing history of arterial hypertension diagnosed in 2008, currently managed with an ACE inhibitor and a calcium channel blocker. Type 2 diabetes mellitus was diagnosed in 2015 and is managed with oral antidiabetic agents and insulin. Additionally, the patient has a history of hypercholesterolemia and is on statin therapy. There is no known history of malignancy, significant surgical procedures, or major allergies.

Family history is notable for coronary artery disease in the patient's father, who suffered a myocardial infarction at age 62. The patient is a former smoker with a 20 pack-year history, having ceased smoking in 2010. Alcohol consumption is reported as moderate.

Follow-up Recommendations:

We recommend the following follow-up measures: (1) General practitioner visit within 2 weeks for medication review and blood pressure control. (2) Laboratory workup including complete blood count, renal function, liver enzymes, and HbA1c in 3 months. (3) Specialist referral as outlined above. (4) Repeat imaging in 6 months if clinically indicated.

The patient has been fully informed of the diagnosis, treatment plan, and recommended follow-up schedule. Written discharge instructions were provided. The patient expressed understanding and willingness to comply with the recommended measures.

Medication on Discharge:

The following medications were prescribed at discharge: metformin 1000 mg twice daily, ramipril 5 mg once daily, atorvastatin 40 mg once daily, aspirin 100 mg once daily, and pantoprazole 40 mg once daily. The patient

was counseled on the importance of adherence and potential side effects.

Prof. Martin Schuler, MD

Director, Oncology